

Name : **MR. HIMANK KANDPAL**

Age/ Gender : 24 years / Male

Sample Type : Serum

Sample ID : MB2236343

Client Name : Phasorz pvt ltd - GZB010

Ref. Doctor : SELF

Collected : 30/07/2026, 09:44 AM

Received : 30/07/2026, 01:00 PM

Reported : 30/07/2026, 02:34 PM

**BIOCHEMISTRY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Kidney Function Test 2-Mini (KFT) - Corporate

Creatinine (Method: Serum, Alkaline picrate-kinetic)	0.88	mg/dL	0.67 - 1.17
Urea (Method: Urease)	24.09	mg/dL	Adults: 17- 43
Uric Acid (Method: Serum, Uricase, Colorimetric)	7.02	mg/dL	3.5 - 7.2
Blood Urea Nitrogen (Method: Serum, Urease)	11.26	mg/dL	7 - 18
BUN/Creatinine (Method: calculated)	12.80	Ratio	10 - 20
Urea/Creatinine (Method: calculated)	27.38	Ratio	40 - 100
Glomerular Filtration Rate (eGFR) (Method: Calculated)	123.14	mL/min	90 - 120 mL/min/1.73 m ²

Note

Report value checked, Kindly correlate clinically.

Interpretation:

Creatinine: Muscles produce creatinine, a waste product, from creatine phosphate, a substance that stores a lot of energy. Unlike urea, the amount of creatinine generated is constant and mostly depends on muscle mass. Age, gender, race, muscularity, exercise, pregnancy, and several other physiological characteristics can all have an impact on serum creatinine levels. Decreased serum Creatinine is associated with increasing Age and poor muscle mass, such as muscular atrophy. Both acute and chronic renal disease and blockage are associated with elevated blood creatinine levels. Creatinine is not an appropriate indicator for identifying kidney disease in its early stages since an increase in blood creatinine is only seen when there is significant nephron damage.

High **Urea, Uric Acid, and Blood Urea Nitrogen (BUN)** could indicate poor renal function, in addition to other etiologies

Dr. Tarun Mittal
(Consultant Pathologist)

NOTE : This sample is processed at HealthMap Diagnostics Pvt Ltd (8, Basant Rd, Qureshi Market, Pocket E, Nehru Nagar II, Nehru Nagar, Ghaziabad, Uttar Pradesh, India) Lab.

Name : **MR. HIMANK KANDPAL**

Age/ Gender : 24 years / Male

Sample Type : EDTA WB

Sample ID : MB2236343

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Ref. Doctor : SELF

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**HAEMATOLOGY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Complete Blood Count; CBC (EDTA whole blood)**Erythrocytes** (Whole Blood)

Hemoglobin (Hb) (Method: NonCyanmethemoglobin Photometric Measurement)	14.9	gm/dL	13.5 - 18.0
Erythrocyte (RBC) Count (Method: Electrical Impedence)	4.99	mil/cu.mm	4.7 - 6.0
Packed Cell Volume(Hematocrit) (Method: Calculated)	44.1	%	42 - 52
Mean Cell Volume (MCV) (Method: Electrical Impedence)	88.4	fL	78 - 100
Mean Cell Haemoglobin (MCH) (Method: Calculated)	29.8	pg	27 - 31
Mean Corpuscular Hb Concn (MCHC) (Method: Calculated)	33.7	gm/dL	32 - 36
Red Cell Distribution Width (RDW)-CV (Method: Electrical Impedence)	11.9	%	11.5 - 14.0
Red Cell Distribution Width (RDW)-SD	40.6	fL	40.0 - 55.0
RBC Morphology	Normocytic normochromic		

Leucocytes (Whole, Blood)

Total Leucocytes (WBC) Count (Method: Electrical Impedence)	5150	cell/cu.mm	4000-10000
Neutrophils (Method: VCSn Technology)	42	%	40 - 80
Lymphocytes (Method: VCSn Technology)	46	%	20 - 40
Monocytes (Method: VCSn Technology)	9	%	2 - 10
Eosinophils (Method: VCSn Technology)	3	%	1 - 6
Basophils (Method: VCSn Technology)	0	%	1-2

Absolute Count

Absolute Neutrophil Count (Method: Calculated)	2.16	* 10 ⁹ /L	2.0 - 7.0
Absolute Lymphocyte Count (Method: Calculated)	2.37	* 10 ⁹ /L	1-3
Absolute Monocyte Count (Method: Calculated)	0.46	* 10 ⁹ /L	0.2-1.0
Absolute Eosinophil Count (Method: Calculated)	0.15	* 10 ⁹ /L	0.0-0.5
Absolute Basophils Count (Method: Calculated)	0	* 10 ⁹ /L	1-2
WBC Morphology	Relative lymphocytosis		

Platelets (Whole, Blood)

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HAEMATOLOGY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Platelet Count (Method: Electrical Impedence)	221	10 ³ /ul	150 - 450
Mean Platelet Volume (MPV) (Method: Electrical Impedence)	7.9	fL	7.2 - 11.7
Platelet Morphology	Adequate on smear		
PCT (Method: Calculated)	0.174	%	0.2 - 0.5
PDW (Method: Calculated)	9.8	%	9.0 - 17.0
Platelet large cell ratio (P-LCR)	23.2	%	15 - 35

Tests done on Automated Five Part Cell Counter. (WBC, RBC, Platelet count by impedance method, colorimetric method for Hemoglobin, WBC differential by flow cytometry using laser technology other parameters are calculated). All Abnormal Haemograms are reviewed confirmed microscopically.

FAST. ACCURATE. RELIABLE

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Name : **MR. HIMANK KANDPAL**

Age/ Gender : 24 years / Male

Sample Type : Plasma - F

Sample ID : MB2236343F

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**BIOCHEMISTRY**

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Glucose Fasting (F)**Glucose Fasting***

(Method: Plasma, Hexokinase)

87.65

mg/dL

Normal: 70-100

Impaired Fasting Glucose (IFG):
100-125Diabetes Mellitus: ≥ 126

(On more than one occasion)

(American Diabetes Association
guidelines 2017)

manipalTRUtest

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Name : **MR. HIMANK KANDPAL**

Age/ Gender : 24 years / Male

Sample Type : Serum

Sample ID : MB2236343

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**BIOCHEMISTRY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Lipid Profile 2, Basic

Cholesterol Total (Method: Serum, Cholesterol oxidase esterase, peroxidase)	177.99	mg/dL	Desirable: ≤ 200 Borderline High: 201-239 High: > 240 Ref: The National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Triglycerides (Method: Serum, GOD-POD)	289.62	mg/dL	Normal: < 150 Borderline High: 150-199 High: 200-499 Very High: ≥ 500
Cholesterol - HDL (Method: Serum, Direct measure-PEG)	27.63	mg/dL	Normal: > 40 Major Heart Risk: < 40
Cholesterol - LDL (Method: Serum, calculated)	92.44	mg/dL	Optimal: < 100 Near optimal/above optimal: 100-129 Borderline high: 130-159 High: 160-189 Very High: ≥ 190
Cholesterol VLDL (Method: Serum, calculated)	57.92	mg/dL	6 - 38
Cholesterol - Non-HDL (Method: Calculated)	150.36	mg/dL	< 130 Desirable
Total cholesterol / HDL (Method: Calculated)	6.44	Ratio	0 - 5.0
LDL / HDL (Method: Calculated)	3.35	Ratio	0 - 3.5
HDL / LDL (Method: Calculated)	0.30	Ratio	0 - 3.5

Comments:

Report value checked, Kindly correlate clinically.

Interpretation:

- For non-fasting samples**, the biological reference interval remains the same for all parameters, except for triglyceride as cholesterol (HDL, LDL, total), which changes only by a small amount in the non-fasting state; **the recommended desired value for triglycerides is <175 mg/dL**. In the non-fasting state, **individuals with a non-fasting triglyceride level >200 mg/dL, are recommended to perform a follow-up fasting lipid panel in 2 to 4 weeks.**
- As per the consensus of the Lipid Association of India, Non-HDL cholesterol and LDL cholesterol can be used as targets to monitor the effectiveness of lipid-lowering therapy.

Associated tests: Apolipoproteins A1, Apolipoproteins B, Apolipoprotein B/A1 Ratio, Lipoprotein(a)**NOTE :** This sample is processed at HealthMap Diagnostics Pvt Ltd (8, Basant Rd, Qureshi Market, Pocket E, Nehru Nagar II, Nehru Nagar, Ghaziabad, Uttar Pradesh, India) Lab.

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**BIOCHEMISTRY**

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Estimated glomerular filtration rate (EGFR)

eGFR (CKD-EPI)	123.14	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
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**BIOCHEMISTRY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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SGOT

SGOT*

(Method: UV with P5P, IFCC 37 Degree)

32.81

U/L

<50

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**BIOCHEMISTRY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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SGPT

SGPT*

(Method: UV with P5P, IFCC 37 Degree)

73.88

U/L

<50

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**IMMUNOLOGY**

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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TSH (Thyroid Stimulating Hormone), Ultrasensitive

TSH Ultra* (Method: CLIA)	1.57	μIU/mL	New born - 4days :1.0 - 39.0 2 - 20wk : 1.7 - 9.1 21wk - 20yr : 0.7 - 6.4 Adults 21yr - 54yr : 0.4 - 4.2 55yr - 87yr : 0.5 - 8.9
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1. Serum TSH concentrations exhibit a diurnal variation with the peak occurring during the night.
2. Useful for: Screening for thyroid dysfunction and detecting mild (subclinical), as well as overt, primary hypo- or hyperthyroidism in ambulatory patients.
3. Monitoring patients on thyroid replacement therapy.
4. Confirmation of thyroid-stimulating hormone (TSH) suppression in thyroid cancer patients on thyroxine suppression therapy.
5. Prediction of thyrotropin-releasing hormone-stimulated TSH response.

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IMMUNOLOGY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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Vitamin B12; Cyanocobalamin

Vitamin B12-Cyanocobalamin*

(Method: Serum, CLIA)

136.1

pg/ml

120 - 914

Interpretation:

LOW:

Patients with serum B12 levels between 150 and 210 pg/mL are considered borderline and should be evaluated further by functional tests for Vitamin b12 deficiency. The plasma homocysteine level is a good screening test. A normal level effectively exclude Vitamin b12 and folate deficiency in an asymptomatic patient.

NORMAL:

Vitamin b12(cobalamin) is necessary for hematopoiesis and normal neuronal function. In humans, it is obtained only from animal proteins and requires intrinsic factor (IF) for absorption. The body uses its Vitamin b12 stores very economically, reabsorbing Vitamin b12 from the ileum and returning it to the liver. very little is excreted.

High:

Patients taking Vitamin b12 supplementation may have misleading results. Your healthcare provider will advise you on which ones to stop taking. High levels of Vitamin b12 are uncommon and not usually clinically monitored. However, if someone has a condition such as chronic myeloproliferative neoplasm, diabetes, heart failure, obesity, AIDS, or severe liver disease, then that person may have an increased Vitamin b12 level. Many other conditions are known to cause an increase in the serum Vitamin b12 concentration including: Ingestion of vitamin C, Ingestion of estrogens, Ingestion of Vitamin A, Hepatocellular injury, myeloproliferative disorder, Uremia.

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